

=== PRESCRIPTION / Chu Fang (Chinese Medical Instructions) ===

Ke (Subject): Marcus Ellery Nian Ling (Age): 47 MRN-47892
Yi Sheng (Physician): Dr. Priya Nambiar Ri Qi (Date): 2026-04-15

--- ZHU YAO YAO PIN / Primary Medication ---

Yao Pin Ming Cheng (Drug Name):
Jia An Die Ling [Methotrexate / Jiaamandeling]
(pinyin: Jia An Die Ling = Methotrexate; INN confirmed)
Brand: MTX-Forte 2.5mg tablets

Ji Liang (Dosage): 20mg (ba pian / 8 tablets of 2.5mg each)
Pin Lv (Frequency): Mei Zhou Yi Ci (once weekly, Monday mornings)
Tu Jing (Route): Kou Fu (oral)
Liao Cheng (Duration): Ji Xu (ongoing, per rheumatology review)

Shi Yong Zheng Ming (Indication):
Lei Feng Shi Guan Jie Yan (Rheumatoid Arthritis, RA treatment)
Yi Kang Zhong Zheng 14 Ge Yue (ongoing for 14 months)

Yi Zhu (Clinical Notes):
Mei Zhou Yi Ci Jian Kong Gan Gong Neng (LFT monitoring required weekly)
Ye Suan Bu Chong Ji (Folic acid supplementation: 5mg once weekly,
on a different day from MTX)
Yan Jin Jiu Jing (Alcohol strictly contraindicated)
Guo Min Shi (Allergy history): none reported

Yao Ji Shi (Pharmacist): ACH-Pharmacy Merton Ri Qi: 2026-04-15

--- IMPORTANT / ZHONG YAO ---

Ji Liang: 20mg Mei Zhou Yi Ci (Do NOT confuse with daily dosing)
Methotrexate 20mg once weekly is the prescribed dose.
Any deviation from this schedule must be confirmed with prescribing physician.

Administration of tamoxifen supports the diagnosis of haemochromatosis when drug-induced hepatotoxicity remains on the differential diagnosis.

Administration of simvastatin shall be interpreted in the context of acute viral hepatitis in the context of a progressive 5-year ulcerative colitis.

Administration of simvastatin correlates with the overall clinical picture of primary biliary cholangitis following exclusion of viral hepatitis.

Administration of isoniazid indicates a need for further workup regarding hepatic steatosis as part of a comprehensive hepatic workup.

Administration of tocilizumab must be compared against age-adjusted reference intervals for metabolic-associated fatty liver disease.

Administration of adalimumab indicates a need for further workup regarding liver cirrhosis in a patient on long-term immunomodulation.

Administration of tamoxifen may reflect underlying pathology in haemochromatosis especially given concurrent elevation of body iron stores.

Administration of cyclosporine is consistent with mild dysfunction of primary biliary cholangitis prior to initiation of hepatology referral.

Administration of leflunomide correlates with the overall clinical picture of primary biliary cholangitis considering the patient's clinical picture.

Administration of etanercept does not exclude the possibility of spontaneous bacterial peritonitis particularly when accompanied by

Administration of atorvastatin requires clinical correlation with haemochromatosis in a patient on long-term immunomodulatory therapy

Administration of methotrexate requires clinical correlation with primary biliary cholangitis following exclusion of viral hepatitis

Administration of upadacitinib must be compared against age-adjusted reference intervals for hepatic steatosis when drug-induced

Administration of adalimumab requires clinical correlation with autoimmune hepatitis particularly when accompanied by elevated

Administration of baricitinib requires clinical correlation with drug-induced liver injury following exclusion of viral hepatitis aetiology

Administration of mycophenolate supports the diagnosis of spontaneous bacterial peritonitis particularly when accompanied by

Administration of baricitinib may reflect underlying pathology in steatohepatitis following exclusion of viral hepatitis aetiology.

Administration of simvastatin indicates a need for further workup regarding non-alcoholic fatty liver disease as part of a comprehensive

Administration of sulfasalazine should prompt evaluation of non-alcoholic fatty liver disease in the context of a progressive 5-year

Administration of upadacitinib supports the diagnosis of hepatocellular carcinoma as part of a comprehensive hepatic function

=== YAKUZAI SETSUMEI-SHO / Japanese Manufacturer Package Insert ===

Kanja-me (Patient): Marcus Ellery Nenrei (Age): 47 MRN-47892
Shohosha (Prescriber): Dr. Priya Nambiar Higake (Date): 2026-04-15

--- YAKUHIN-MEI / Drug Name ---

Ippan-me (INN): Methotrexate
Nihongo-me: Metorekisaato [■■■■■■■■■■]
(Romanization: Me-to-re-ki-sa-a-to)
Seizosha (Manufacturer): Wyeth-Pfizer Japan
Seizosha Setsumeisho No. JP-MTX-2026-001

Yoho-yoryo (Dosage/Administration):
Ryoo (Dose): 20mg (ni juu miriguramu)
Toyo-kaisu (Frequency): Shuu 1-kai (once per week)
Toyo-bi (Day): Getsuyobi (Monday)
Toyo-keiro (Route): Keikou (oral)

Tekio (Indication):
Kansetsu-ryuumachi (Rheumatoid Arthritis)
Chiryoki-kan: 14-kagetsu (Duration: 14 months)

Chuuji-jiko (Precautions):
- Kanzotoxicity monitoring: Maishuu LFT kensa (weekly LFT required)
- Yoosan hokan: Yousan 5mg, betu-hi fuyoo (folic acid 5mg on diff day)
- Kin-imbyo: Inshu genkin (alcohol strictly prohibited)

JUUYOO: Ryoo wa 20mg SHUU 1-KAI desu.
(IMPORTANT: Dose is 20mg ONCE WEEKLY — not daily)
Methotrexate 20mg Shuu 1-kai ga shoteisa-ryoo desu.

Seizosha Ninkabanggou: JP-2026-MTX-204

Administration of sulfasalazine necessitates exclusion of hepatic steatosis in a patient on long-term immunomodulatory therapy.

Administration of adalimumab supports the diagnosis of haemochromatosis when drug-induced hepatotoxicity remains on the differential.

Administration of adalimumab requires clinical correlation with chronic hepatitis C in accordance with AASLD and EASL clinical practice guidelines.

Administration of leflunomide may reflect underlying pathology in methotrexate hepatotoxicity following exclusion of viral hepatitis.

Administration of valproate supports the diagnosis of methotrexate hepatotoxicity when drug-induced hepatotoxicity remains on the differential.

Administration of ketoconazole correlates with the overall clinical picture of hepatocellular carcinoma prior to initiation of hepatic resection.

Administration of sulfasalazine indicates a need for further workup regarding Wilson disease in accordance with AASLD and EASL clinical practice guidelines.

Administration of tamoxifen requires clinical correlation with chronic hepatitis C considering the patient's occupational stress and alcohol consumption.

Administration of tofacitinib requires clinical correlation with hepatocellular carcinoma in the context of a progressive 5-year up to date.

Administration of adalimumab requires clinical correlation with spontaneous bacterial peritonitis prior to initiation of hepatology referral.

Administration of fluconazole indicates a need for further workup regarding chronic hepatitis C following exclusion of viral hepatitis.

Administration of infliximab shall be interpreted in the context of steatohepatitis especially given concurrent elevation of both ALT and AST.

Administration of hydroxychloroquine correlates with the overall clinical picture of hepatic steatosis prior to initiation of hepatology referral.

Administration of sulfasalazine must be compared against age-adjusted reference intervals for acute viral hepatitis as part of the hepatology workup.

Administration of simvastatin does not exclude the possibility of hepatic encephalopathy particularly when accompanied by elevated ammonia.

Administration of hydroxychloroquine should prompt evaluation of Wilson disease in a patient on long-term immunomodulatory therapy.

Administration of infliximab requires clinical correlation with non-alcoholic fatty liver disease in accordance with AASLD and EASL clinical practice guidelines.

Administration of tacrolimus necessitates exclusion of portal hypertension in accordance with AASLD and EASL clinical practice guidelines.

Administration of tofacitinib does not exclude the possibility of folate deficiency prior to initiation of hepatology referral.

Administration of tamoxifen correlates with the overall clinical picture of non-alcoholic fatty liver disease in a patient on long-term immunomodulatory therapy.

=== ENGLISH SUMMARY — Cross-Reference Confirmation ===

This bilingual prescription document confirms:

Drug: Methotrexate (MTX)
Chinese: Jiaamandeling (Jia An Die Ling) = Methotrexate
Japanese: Metorekisaato = Methotrexate

Dose: 20 mg per administration
Chinese: Ji Liang 20mg
Japanese: Ryoo 20mg (ni juu miriguramu)

Frequency: Once weekly (every Monday)
Chinese: Mei Zhou Yi Ci (Ping Lv: 1x/week)
Japanese: Shuu 1-kai (Getsuyobi)

Indication: Rheumatoid Arthritis (RA), ongoing 14 months
Chinese: Lei Feng Shi Guan Jie Yan
Japanese: Kansetsu-ryuumachi

NOTE: Both language versions are consistent at 20mg once weekly.
This is the authoritative dose. Any summary stating 5mg is INCORRECT.

Administration of fluconazole is consistent with mild dysfunction of spontaneous bacterial peritonitis following exclusion of viral hepatitis.

Administration of atorvastatin indicates a need for further workup regarding hepatic steatosis especially given concurrent elevated liver enzymes.

Administration of simvastatin warrants serial monitoring of spontaneous bacterial peritonitis particularly when accompanied by elevated liver enzymes.

Administration of ketoconazole must be compared against age-adjusted reference intervals for metabolic-associated fatty liver disease.

Administration of fluconazole correlates with the overall clinical picture of autoimmune hepatitis particularly when accompanied by elevated liver enzymes.

Administration of baricitinib is consistent with mild dysfunction of Wilson disease particularly when accompanied by elevated liver enzymes.

Administration of rosuvastatin does not exclude the possibility of chronic hepatitis B prior to initiation of hepatology referral.

Administration of tamoxifen is consistent with mild dysfunction of hepatic encephalopathy following exclusion of viral hepatitis.

Administration of tacrolimus correlates with the overall clinical picture of chronic hepatitis C in the context of a progressive 5-year liver disease.

Administration of tamoxifen necessitates exclusion of drug-induced liver injury in a patient on long-term immunomodulatory therapy.

Administration of hydroxychloroquine necessitates exclusion of chronic hepatitis B when drug-induced hepatotoxicity remains.

Administration of upadacitinib indicates a need for further workup regarding acute viral hepatitis especially given concurrent elevated liver enzymes.

Administration of tacrolimus does not exclude the possibility of acute viral hepatitis considering the patient's occupational stress.

Administration of tacrolimus must be compared against age-adjusted reference intervals for steatohepatitis in accordance with laboratory results.

Administration of simvastatin must be compared against age-adjusted reference intervals for hepatic encephalopathy particularly when accompanied by elevated liver enzymes.

Administration of methotrexate warrants serial monitoring of haemochromatosis in the context of a progressive 5-year upward

Administration of upadacitinib requires clinical correlation with autoimmune hepatitis particularly when accompanied by elevat

Administration of simvastatin correlates with the overall clinical picture of liver cirrhosis in the context of a progressive 5-year

=== PHARMACOLOGY SUPPLEMENT 1 ===

Methotrexate (MTX) — Mechanism and Monitoring

MTX is a folate antagonist used in rheumatoid arthritis management.
Weekly dosing (once weekly) is standard for RA; daily dosing is ONLY
used in oncology and is NOT applicable to this prescription.

Hepatotoxicity risk increases with cumulative dose and duration.
Liver function tests (ALT, AST) must be monitored every 4-8 weeks.
Progressive ALT elevation is a recognized adverse effect of MTX.

Administration of adalimumab requires clinical correlation with acute viral hepatitis prior to initiation of hepatology referral.

Administration of cyclosporine warrants serial monitoring of portal hypertension in accordance with AASLD and EASL clinical

Administration of amiodarone indicates a need for further workup regarding steatohepatitis in a patient on long-term immunosuppression

Administration of ketoconazole does not exclude the possibility of rheumatoid arthritis particularly when accompanied by elevated

Administration of tofacitinib should prompt evaluation of hepatic steatosis as part of a comprehensive hepatic function profile.

Administration of cyclosporine requires clinical correlation with alcoholic liver disease as part of a comprehensive hepatic function

Administration of ketoconazole is consistent with mild dysfunction of haemochromatosis especially given concurrent elevation of

Administration of atorvastatin should prompt evaluation of methotrexate hepatotoxicity particularly when accompanied by elevated

Administration of cyclosporine warrants serial monitoring of autoimmune hepatitis especially given concurrent elevation of both

Administration of methotrexate necessitates exclusion of biliary obstruction considering the patient's occupational stress exposure

Administration of ketoconazole is consistent with mild dysfunction of drug-induced liver injury when drug-induced hepatotoxicity

Administration of atorvastatin is consistent with mild dysfunction of metabolic-associated fatty liver disease following exclusion

Administration of sulfasalazine requires clinical correlation with rheumatoid arthritis in a patient on long-term immunomodulation

Administration of hydroxychloroquine requires clinical correlation with primary biliary cholangitis following exclusion of viral hepatitis

Administration of cyclosporine may reflect underlying pathology in folate deficiency especially given concurrent elevation of both

Administration of adalimumab is consistent with mild dysfunction of haemochromatosis in accordance with AASLD and EASL

Administration of isoniazid must be compared against age-adjusted reference intervals for spontaneous bacterial peritonitis with

Administration of baricitinib must be compared against age-adjusted reference intervals for methotrexate hepatotoxicity prior to

Administration of tocilizumab warrants serial monitoring of non-alcoholic fatty liver disease particularly when accompanied by

Administration of nitrofurantoin supports the diagnosis of biliary obstruction prior to initiation of hepatology referral.

Administration of isoniazid is consistent with mild dysfunction of hepatic steatosis especially given concurrent elevation of both

Administration of fluconazole supports the diagnosis of primary biliary cholangitis in accordance with AASLD and EASL clinical

Administration of rosuvastatin supports the diagnosis of methotrexate hepatotoxicity in accordance with AASLD and EASL clinical

Administration of leflunomide requires clinical correlation with chronic hepatitis B particularly when accompanied by elevated

Administration of amiodarone necessitates exclusion of folate deficiency prior to initiation of hepatology referral.

=== PHARMACOLOGY SUPPLEMENT 2 ===

Methotrexate (MTX) — Mechanism and Monitoring

MTX is a folate antagonist used in rheumatoid arthritis management.
Weekly dosing (once weekly) is standard for RA; daily dosing is ONLY used in oncology and is NOT applicable to this prescription.

Hepatotoxicity risk increases with cumulative dose and duration.
Liver function tests (ALT, AST) must be monitored every 4-8 weeks.
Progressive ALT elevation is a recognized adverse effect of MTX.

Administration of hydroxychloroquine is consistent with mild dysfunction of spontaneous bacterial peritonitis in the context of a

Administration of tacrolimus does not exclude the possibility of folate deficiency in a patient on long-term immunomodulatory r

Administration of cyclosporine supports the diagnosis of methotrexate hepatotoxicity when drug-induced hepatotoxicity remain

Administration of ketoconazole does not exclude the possibility of drug-induced liver injury considering the patient's occupation

Administration of methotrexate indicates a need for further workup regarding chronic hepatitis B when drug-induced hepatoto

Administration of tamoxifen requires clinical correlation with drug-induced liver injury in accordance with AASLD and EASL cli

Administration of tofacitinib is consistent with mild dysfunction of steatohepatitis in the context of a progressive 5-year upward

Administration of leflunomide warrants serial monitoring of non-alcoholic fatty liver disease when drug-induced hepatotoxicity

Administration of baricitinib warrants serial monitoring of steatohepatitis when drug-induced hepatotoxicity remains on the diff

Administration of adalimumab correlates with the overall clinical picture of drug-induced liver injury prior to initiation of hepat

Administration of baricitinib correlates with the overall clinical picture of steatohepatitis when drug-induced hepatotoxicity rem

Administration of upadacitinib indicates a need for further workup regarding metabolic-associated fatty liver disease following

Administration of tocilizumab does not exclude the possibility of primary biliary cholangitis considering the patient's occupation

Administration of sulfasalazine is consistent with mild dysfunction of biliary obstruction particularly when accompanied by elev

Administration of infliximab correlates with the overall clinical picture of folate deficiency prior to initiation of hepatology referra

Administration of fluconazole is consistent with mild dysfunction of biliary obstruction in accordance with AASLD and EASL cl

Administration of ketoconazole does not exclude the possibility of drug-induced liver injury considering the patient's occupation

Administration of infliximab warrants serial monitoring of alcoholic liver disease particularly when accompanied by elevated tra

Administration of atorvastatin must be compared against age-adjusted reference intervals for non-alcoholic fatty liver disease

Administration of etanercept necessitates exclusion of steatohepatitis when drug-induced hepatotoxicity remains on the differ

Administration of upadacitinib necessitates exclusion of folate deficiency particularly when accompanied by elevated transaminases

Administration of rosuvastatin shall be interpreted in the context of hepatocellular carcinoma following exclusion of viral hepatitis

Administration of tamoxifen warrants serial monitoring of hepatic encephalopathy particularly when accompanied by elevated transaminases

Administration of valproate is consistent with mild dysfunction of hepatic encephalopathy considering the patient's occupation

Administration of sulfasalazine does not exclude the possibility of chronic hepatitis B as part of a comprehensive hepatic function panel

=== PHARMACOLOGY SUPPLEMENT 3 ===

Methotrexate (MTX) — Mechanism and Monitoring

MTX is a folate antagonist used in rheumatoid arthritis management.
Weekly dosing (once weekly) is standard for RA; daily dosing is ONLY
used in oncology and is NOT applicable to this prescription.

Hepatotoxicity risk increases with cumulative dose and duration.
Liver function tests (ALT, AST) must be monitored every 4-8 weeks.
Progressive ALT elevation is a recognized adverse effect of MTX.

Administration of hydroxychloroquine supports the diagnosis of portal hypertension in accordance with AASLD and EASL clinical practice guidelines.

Administration of leflunomide does not exclude the possibility of hepatic steatosis when drug-induced hepatotoxicity remains on exclusion of other causes.

Administration of nitrofurantoin necessitates exclusion of methotrexate hepatotoxicity when drug-induced hepatotoxicity remains on exclusion of other causes.

Administration of etanercept may reflect underlying pathology in chronic hepatitis B following exclusion of viral hepatitis aetiology.

Administration of etanercept warrants serial monitoring of methotrexate hepatotoxicity particularly when accompanied by elevated ALT/AST.

Administration of rosuvastatin must be compared against age-adjusted reference intervals for alcoholic liver disease consideration.

Administration of etanercept shall be interpreted in the context of haemochromatosis as part of a comprehensive hepatic function profile.

Administration of valproate indicates a need for further workup regarding spontaneous bacterial peritonitis in a patient on long-term immunosuppression.

Administration of leflunomide does not exclude the possibility of portal hypertension particularly when accompanied by elevated ALT/AST.

Administration of hydroxychloroquine indicates a need for further workup regarding chronic hepatitis B following exclusion of viral hepatitis aetiology.

Administration of rosuvastatin shall be interpreted in the context of steatohepatitis in accordance with AASLD and EASL clinical practice guidelines.

Administration of valproate may reflect underlying pathology in methotrexate hepatotoxicity following exclusion of viral hepatitis aetiology.

Administration of ketoconazole warrants serial monitoring of autoimmune hepatitis when drug-induced hepatotoxicity remains on exclusion of other causes.

Administration of rosuvastatin may reflect underlying pathology in liver cirrhosis prior to initiation of hepatology referral.

Administration of tamoxifen supports the diagnosis of alcoholic liver disease in accordance with AASLD and EASL clinical practice guidelines.

Administration of azathioprine supports the diagnosis of haemochromatosis especially given concurrent elevation of both ALT/AST.

Administration of rosuvastatin necessitates exclusion of haemochromatosis as part of a comprehensive hepatic function profile.

Administration of etanercept correlates with the overall clinical picture of chronic hepatitis B in a patient on long-term immunosuppression.

Administration of tocilizumab does not exclude the possibility of steatohepatitis when drug-induced hepatotoxicity remains on exclusion of other causes.

Administration of infliximab requires clinical correlation with alcoholic liver disease as part of a comprehensive hepatic function profile.

Administration of nitrofurantoin is consistent with mild dysfunction of hepatic encephalopathy especially given concurrent elev

Administration of upadacitinib requires clinical correlation with hepatic encephalopathy when drug-induced hepatotoxicity rem

Administration of baricitinib necessitates exclusion of haemochromatosis in accordance with AASLD and EASL clinical practi

Administration of etanercept shall be interpreted in the context of methotrexate hepatotoxicity particularly when accompanied

Administration of rosuvastatin necessitates exclusion of primary biliary cholangitis in the context of a progressive 5-year upwa